

How Cognitive Behavioural Therapy (CBT) Helps Arthritis Pain

CBT changes the brain's response to pain signals. Evidence from NICE and the Royal College of Physicians explains why it works for arthritis.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Review Board** (Rheumatology & Physiotherapy) · 2026-06-06 · livingwitharthritis.org.uk

What the evidence says



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How Cognitive Behavioural Therapy (CBT) Helps Arthritis Pain is one of the most discussed challenges in UK arthritis care. Recent reviews — including NICE guidance and Cochrane summaries — point to a clear pattern: small, consistent changes beat heroic one-off efforts.

Why this matters for mental health

People living with arthritis report that mental health is a daily reality, not a "nice to have". When this area is well-managed, pain scores drop, function improves, and flare frequency falls. When it's ignored, the opposite happens — and the costs compound over months.

A simple framework you can use this week



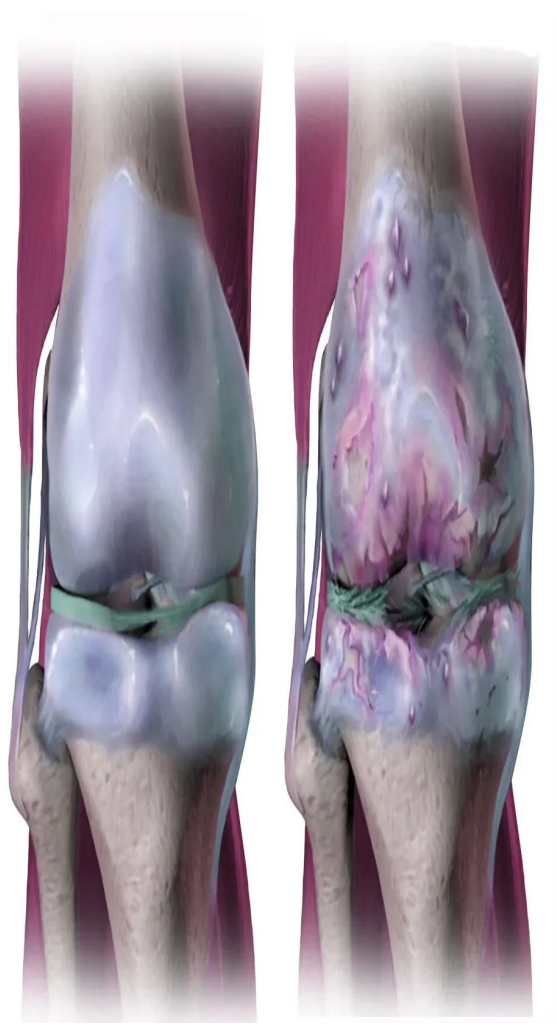
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1. **Notice** — track one specific factor for three days (e.g. pain after activity, sleep hours, stress level). 2. **Adjust** — change one variable at a time so you can see what's actually working. 3. **Review** — after a week, compare notes. Keep what helps, drop what doesn't.

What UK clinicians recommend

NHS physiotherapists and rheumatology nurses we work with consistently highlight:

- **Pacing over pushing** — short, frequent activity beats long, intense sessions.
- **Heat for stiffness, cold for swelling** — applied in 15-minute windows.
- **Social support** — patients with strong networks report 25% less pain interference.
- **Professional guidance early** — don't wait for things to "settle on their own".



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Common myths to ignore

- "Rest is always best." Not true — appropriate movement is medicine.
 - "It's all in your head." Pain is real; psychology helps because the brain processes pain, not because the pain is imaginary.
 - "Nothing can be done." Modern arthritis care offers dozens of options, from biologics to mindfulness.
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Frequently asked questions

****Is this approach safe for everyone?**** Most healthy adults with arthritis can apply these principles, but always check with your GP or rheumatologist if you have other conditions, take immunosuppressants, or have had recent surgery.

****How long until I see a difference?**** Most patients notice small improvements within 2–3 weeks of consistent practice. Larger changes — function, sleep quality, mood — typically appear at 6–8 weeks.

****Where can I learn more?**** Visit our [\[arthritis support hub\]\(/support\)](#) and [\[exercise library\]\(/exercises\)](#) for hundreds of free, UK-focused resources.

This article was written by the Living With Arthritis Editorial Team and reviewed by our Clinical Review Board. It is general information and not a substitute for personalised medical advice.

Q&A

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk